

23STCV23235 23STCV23235

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Case Number: 23STCV23235 Hearing Date: July 21, 2026 Dept: 513

Superior Court of California

County of Los Angeles – Central District

Department

513

PERLA SUSANA LEON

, by and through the Estate Administrator Diana Viteri Perez, et al.;

Plaintiffs,

vs.

ALVARADO

TERRACE CARE CENTER, LLC d/b/a ALVARADO CARE CENTER

, et al.,

Defendants.

Case

No.:

23STCV23235

Hearing

Date:

July
21, 2026

Time:

10:00 a.m.

[Tentative]

Order RE:

motion for summary judgment or, in the
alternative, summary adjudication

MOVING PARTY: Defendants Alvarado Terrace Care
Center, LLC, d/b/a Alvarado Terrace Care Center (the "Facility"), Pacificare
Health Management, LLC, Helene Mayer 2007 Irrevocable Trust, f/b/o Aaron Mayer
12/29/06, Helene Mayer 2007 Irrevocable Trust, f/b/o Abraham Mayer 12/29/06, Helene
Mayer 2007 Irrevocable Trust, f/b/o Akiva Mayer 12/29/06, Helene Mayer 2007
Irrevocable Trust, f/b/o Aviva Mayer 12/29/06, Helene Mayer 2007 Irrevocable
Trust, f/b/o Talia Mayer 12/29/06, and Helene Mayer 2007 Irrevocable Trust,
f/b/o Zachary Mayer 12/29/06 (collectively, "Defendants")

RESPONDING

PARTY: Plaintiffs

Perla Susana Leon, by and through the Estate Administrator Diana Viteri Perez
("Leon"), and the Estate of Perla Susana Leon, by and through the Estate
Administrator Diana Viteri Perez ("Plaintiffs")

Motion for Summary Judgment or, in the Alternative, Summary
Adjudication

The court
considered the moving, opposition, and reply papers filed in connection with
this motion.

LEGAL STANDARD

The purpose of a motion for summary judgment or summary

adjudication “is to provide courts with a mechanism to cut through the parties’ pleadings in order to determine whether, despite their allegations, trial is in fact necessary to resolve their dispute.”

(Aguilar v. Atlantic Richfield Co.

(2001) 25 Cal.4th 826, 843.) “Code

of Civil Procedure section 437c, subdivision (c), requires the trial judge to grant summary judgment if all the evidence submitted, and ‘all inferences reasonably deducible from the evidence’ and uncontradicted by other inferences or evidence, show that there is no triable issue as to any material fact and that the moving party is entitled to judgment as a matter of law.” (Adler v. Manor Healthcare Corp. (1992) 7 Cal.App.4th 1110, 1119.)

“On a motion for summary judgment, the initial burden is always on the moving party to make a prima facie showing that there are no triable issues of material fact.” (Scalf v. D.B. Log Homes, Inc. (2005) 128 Cal.App.4th 1510, 1519.) A defendant or cross-defendant moving for summary judgment or summary adjudication “has met his or her burden of showing that a cause of action has no merit if the party has shown that one or more elements of the cause of action . . . cannot be established, or that there is a complete defense to the cause of action.” (Code Civ. Proc.,

§ 437c, subd. (p)(2).) “Once the defendant or cross-defendant has met that burden, the burden shifts to the plaintiff or cross-complainant to show that a triable issue of one or more material facts exists as to the cause of action or a defense thereto.” (Code of Civ. Proc., § 437c, subd. (p)(2).) “If the plaintiff cannot do so,

summary judgment should be granted.” (Avivi v. Centro Medico Urgente Medical Center (2008) 159 Cal.App.4th 463, 467.) “When deciding whether to grant summary judgment, the court must consider all of the evidence set forth in the papers (except evidence to which the court has sustained an objection), as well as all reasonable inferences that may be drawn from that evidence, in the light most favorable to the party opposing summary judgment.” (Id. at p. 467; Code of Civ. Proc., § 437c, subd. (c).)

DISCUSSION

1. First

Cause of Action for Elder Abuse

a. Abuse

or Neglect

Under the Elder Abuse and Dependent Adult Civil Protection Act,
“[a]buse of an elder or a dependent adult means any of the following: (1) Physical abuse, neglect, abandonment, isolation, abduction, or other treatment with resulting physical harm or pain or mental suffering. (2) The deprivation by a care custodian of goods or services that are necessary to avoid physical harm or mental suffering.” (Welf. & Inst. Code, §15610.07, subd. (a)(1)-(3).)

“Neglect” is defined as “(1) The negligent failure of any person having the care or custody of an elder or a dependent adult to exercise that degree of care that a reasonable person in a like position would exercise. (2) The negligent failure of an elder or dependent adult to exercise that degree of self care that a reasonable person in a like position would exercise.”
(Welf. & Inst. Code, §15610.57, subd. (a).)

“‘Neglect’ includes, but is not limited to, all of the following: (1) Failure to assist in personal hygiene, or in the provision of food, clothing, or shelter. (2) Failure to provide medical care for physical and mental health needs...(3) Failure to protect from health and safety hazards. (4) Failure to prevent malnutrition or dehydration...” (Welf. & Inst. Code, §15610.57, subd. (b)(1)-(4).)

“Thus, neglect within the meaning of former section 15610.57 appears to cover an area of misconduct distinct from ‘professional negligence’ in section 15657.2: ‘neglect’ as defined in former section 15610.57 and used in section 15657 does not refer to the performance of medical services in a manner inferior to the knowledge, skill and care ordinarily possessed and employed by members of the profession in good standing, but rather to the failure of those responsible for attending to the basic needs and comforts of elderly or dependent adults, regardless of their professional standing, to carry out their custodial obligations.” (Delaney v. Baker (1999) 20 Cal.4th 23, 34.)

“As used in the Act, neglect refers not to the substandard performance of medical services but, rather, to the failure of those responsible for attending to the basic needs and comforts of elderly or dependent adults, regardless of their professional standing, to carry out their custodial obligations. Thus, the statutory definition of ‘neglect’ speaks not of the undertaking of medical services, but of the failure to provide medical care.” (Covenant Care, Inc. v. Supr. Ct.

(2004) 32 Cal.4th 771, 783 [“statutory elder abuse may include the egregious withholding of medical care for physical and mental health needs”].)

“The difficulty in distinguishing between ‘neglect’ and ‘professional negligence’ lies in the fact that some health care institutions, such as nursing homes, perform custodial functions and provide professional medical care.” (Delaney, *supra*, 20 Cal.4th at p. 34.) “Section 15657 provides the way out of this ambiguity: if the neglect is ‘reckless,’ or done with ‘oppression, fraud or malice,’ then the action falls within the scope of section 15657 and as such cannot be considered simply based on professional negligence within the meaning of section 15657.2.” (Id. at p. 35; *Carter v. Prime Healthcare Paradise Valley LLC* (2011) 198 Cal.App.4th 396, 405 [recovery of enhanced remedies under Elder Abuse Act requires plaintiff to prove “by clear and convincing evidence that the defendant has been guilty of recklessness, oppression, fraud, or malice in the commission of the neglect”].)

“‘Recklessness’ refers to a subjective state of culpability greater than simple negligence, which has been described as a ‘deliberate disregard’ of the ‘high degree of probability’ that an injury will occur. Recklessness, unlike negligence, involves more than ‘inadvertence, incompetence, unskillfulness, or a failure to take precautions’ but rather rises to the level of a ‘conscious choice of a course of action ... with knowledge of the serious danger to others involved in it.’” (Delaney, *supra*, 20 Cal.4th at pp. 31-32.)

In *Carter v. Prime Healthcare Paradise Valley LLC* (2011) 198 Cal.App.4th 396, 406-407, the Court of Appeal set forth several factors that must be present for conduct to constitute neglect under Welfare and Institution Code section 15610.57 subdivision (a):

“The plaintiff must allege facts establishing that the defendant (1) had responsibility for meeting the basic needs of the elder or dependent adult, such as nutrition, hydration, hygiene or medical care; (2) knew of conditions that made the elder or dependent adult unable to provide for his or her own basic needs; and (3) denied or withheld goods or services necessary to meet the elder or dependent adult’s basic needs, either with knowledge that injury was substantially certain to befall the elder or dependent adult (if the plaintiff alleges oppression, fraud or malice) or with conscious disregard of the high

probability of such injury (if the plaintiff alleges recklessness).” (Carter, supra, 198 Cal.App.4th at pp. 406-407.)

Defendants argue there is no evidence of elder abuse, neglect, recklessness or willful misconduct.

Defendants submit evidence that Leon was admitted for short term stay to the Facility, which ultimately lasted nine days from December 4, 2020 through December 11, 2020. (Defendants’ SSUMF

Nos. 2 to 24, 31; Defendants’ Evidence ISO MSJ (“Defendants’ Evidence”), Exs. B, D, N and O.) Defendants submit

evidence that Leon was admitted to the Facility with a diagnosis of aspiration pneumonia and urinary tract infection.

(Defendants’ SSUMF No. 3; Defendants’ Evidence, Ex. C.) Defendants submit evidence that Leon was discharged from Providence St. John’s with a medication order for ampicillin-sulbactam to be given intravenously every six hours for nine days for community-acquired pneumonia. (Defendants’ SSUMF No. 3; Defendants’ Evidence, Ex. C.)

Defendants submit evidence of the care provided to Leon at the Facility. Defendants’ claim the evidence establishes that (1) Leon was fed and hydrated; (2) a care plan was created for her to address her desire to return to the group home, her cognitive deficits, communication problems, risk for dehydration, tube feedings, risks for falls, depression, risk for potential nutritional problems, pain and bowel/bladder incontinence; (3) she was regularly examined by physicians and subject to lab tests to determine her health status; (4) she received treatment for her pneumonia through administration of amoxicillin; (5) her condition was evaluated multiple times a day, including for breaks in the skin and pressure wounds. (Defendants’ SSUMF Nos. 4-27; Defendants’ Evidence, Exs. D, H (care plans) I and N.)

Defendants submit evidence that they transferred Leon to an acute care facility once her condition began to decline. (Defendants’ SSUMF Nos. 24-29, Exs. D, N.) Defendants submit evidence Leon was received by the acute care facility within one hour and eight minutes of her condition becoming acute. (Defendants’ SSUMF Nos. 27-28.)

Defendants argue Leon’s pressure injuries arose during her 30-hour stay at Good Samaritan Hospital, not during her stay at the Facility. Defendants submit evidence that, at the time

of her admission to the Facility, there were no pressure wounds. (Defendants' SSUMF No. 4; Ex. D.) Defendants maintain Leon suffered from no

pressure wounds at the time she was transferred to Good Samaritan

Hospital. (Defendants' SSUMF No. 28;

Defendants' Evidence, Ex. N, p. 7 of 11; Ex. S; Ex. U, pp. 10-11; Ex. V, p. 63.)

Defense experts reviewed Leon's medical records, and they opine

that the care and treatment provided at the Facility complied with the standard

of care. (Rosen Dec., ¶48; Hibbs Dec.,

¶48.) Defense expert Hibbs opines that

the Facility administered proper wound care, as Leon did not develop any wounds

while under its care and pressure wounds were first documented on December 13,

2020 by staff at Good Samaritan Hospital.

(Hibbs Dec., ¶51; Defendants' Evidence, Ex. V.) Defense expert Hibbs opines the transfer of

Leon to Good Samaritan Hospital was within the standard of care, if not beyond

it due to the rapidity with which she was transferred. (Id. at ¶52.) Defense expert Hibbs opines Leon was properly

hydrated and fed in accordance with the standard of care, as evidence by the

fact that she gained 5 pounds during her stay at the Facility. (Id. at ¶53.) Defense expert Hibbs also points out that

Leon remained dehydrated despite Good Samaritan's admission of aggressive

intravenous fluids for severe dehydration.

(Id. at ¶54.) Defense

experts opine that Leon died due to her co-morbidities and not the failure to

provide care. (Hibbs Dec., ¶59; Rosen,

¶60.)

However, Defendants failed to provide pin cites to pages of Leon's

medical records to support their claim that (1) a care plan was created and

implemented during her stay at the Facility to address her desire to return to

the group home, her cognitive deficits, communication problems, risk for

dehydration, tube feedings, risks for falls, depression, risk for potential

nutritional problems, pain and bowel/bladder incontinence and (2) the

physician's order regarding her feeding and hydration was implemented and

adhered to during her stay. A review of

the hundreds of pages of medical records does not establish either material

fact.

Defendants attach the care plans as Exhibit H. The care plans attached as Exhibit H have

initiation dates ranging from 12/7/20 to 12/21/20. (Defendants' Evidence, Ex. H.) In particular, the care plan intended to

address Leon's "risk for dehydration or potential fluid deficit r/t tube

feeding" indicates it was initiated on 12/21/2020, after Leon had left the

Facility and after she passed away on 12/18/20.

(Id. at p. 4 of 10.) The

dehydration care plan also lists as an intervention “administer medications as ordered. Monitor/document for side effects and effectiveness.” (Id.) The intervention does not appear to address dehydration.

Defendants attach the physician's order regarding Leon's feeding and hydration schedule as Exhibit G. The

order summary indicates “Enteral Feed Order every 24 hours Enteral Nutrition via Pump Glucerna 1.6 at 65 ml per hour for 20 hours a day via pump per G-tube.

Start Infusion at 7pm and continue until 3p AND every 24 hours every 24 hours

Enteral Nutrition via Pump- Glucerna 1.5 at 55 ml per hour for 20 hours a day via

pump per G-tube. Start ilnfuelon at 7pm and continue until 3p.” (Id. at Ex. G.) The physician also ordered “Enteral Feed Order every 4 hours Give 90 ml sterile water via G-tube every 4 hrs” and

“Enteral Feed Order every shift Flush tube with 20-30ml(cc) of water before and after administration of medication pass.”

(Id.) Defendants claim

they complied with these orders, citing to the expert declarations and Defendants' Exhibits D and G.

The expert declarations are not admissible evidence of the

underlying, percipient facts contained in the medical records. “Expert testimony has its own rules given its issues of reliability because the expert is not testifying to percipient facts,

but instead relies on a multitude of materials, including hearsay, in forming

his or her opinion. Further, the expert is not bound by actual facts in

rendering an opinion, but may opine on hypothetical situations.” (Cooper v. Takeda Pharmaceuticals America, Inc. (2015) 239 Cal.App.4th 555, 583-584.)

The nurses notes attached as Exhibit D also do not establish

compliance with the physician's order, nor do they support SSUMF No. 5. The nurses' notes are from 12/3/20 through 12/11/20. (Defendants' Evidence, Ex.

D.) The notes are general notes

regarding the patient's presentation and, at most, notations that the “G tube is in place” and “running well.” There

are no specific notes regarding administration of Glucerna or water, e.g. when and how much. Exhibit G is the

physician's orders, not evidence of Defendants' compliance with the orders. (Id. at Ex. G.)

Defendants' evidence does not support two key material facts

(SSUMF Nos. 5, 6) pertaining to Defendants' provision for Leon's basic nutritional and hydration needs. Defendants have therefore not met their burden of showing that the first cause of action for elder abuse has no merit.

In opposition, Plaintiffs also argue there was no dehydration care plan, even though the Facility's director of nursing identified dehydration as a risk. (Defendants' Evidence, Ex. H, p. 4 of 10.) Defendants submit Leon's care plans as Exhibit H. The dehydration care plan indicates it was initiated on 12/21/20, after Leon was transferred to Good Samaritan Hospital. (Defendants' Evidence, Ex. H, p. 4 of 10.) The only intervention listed is "administer medications as ordered. Monitor/document for side effects and effectiveness." (Ibid.) Given the plan as written, there was no dehydration care plan while Leon was under the Facility's care. The dehydration care plan only recognizes Leon's risk of dehydration but fails to implement any measures addressing the dehydration. (Ibid.)

The initial physician order dated December 3, 2020 also required (1) 90 mL of sterile water via G-tube every four hours and (2) a 20-30 mL G-tube flush before and after each medicine administration every shift. (Plaintiffs' COE, Ex. G.) Plaintiffs submit the nursing flowsheets as evidence that this protocol was not followed on at least five occasions—December 6, 7, 10 and 11. (Plaintiffs' COE, Ex. O.)

Plaintiffs also point out there is no evidence of tube feedings, including water flushes, on December 7 and 11. (Plaintiffs' Compendium of Evidence, Exs. B, ¶8(p); Ex. C, ¶10(p); Ex. E; Ex. O.) The nursing tube-feeding flowsheets do not reflect consistent compliance with the physician's feeding order. The physician order required Glucerna 1.5 at 55 mL per hour for 20 hours a day via enteral nutrition pump, with the infusion to start at 7:00 p.m. until 3:00 p.m. (Plaintiffs' Compendium of Exhibits, Ex. G.) The nursing flowsheets only document that the Glucerna pump was turned off at 3:00 p.m. on December 4, 5, 6, 8, 9 and 10, but no corresponding documentation that was turned on at 7:00 p.m. (Plaintiffs' Compendium of Exhibits, Ex. B, ¶¶26-29; Ex. C, ¶23; Ex. O, p. 7; Ex. G.)

Plaintiffs' experts also opine it was precisely Defendants' failure to implement a dehydration care plan and to adhere to the physician's orders regarding feeding and hydration that led to Leon's acute decline in health, including severe dehydration. (Plaintiffs' COE, Ex. B, Snodgress Dec., ¶¶28-29; Ex. C, Nasar Dec., ¶¶28-29.) Plaintiffs' experts also opine that Leon's December 7, 2020 lab results were consistent with "active dehydration," and her blood work on December 11, 2020 were consistent with "severe, advanced dehydration and acute kidney injury." (Plaintiffs' COE, Ex. B, ¶¶27, 30-36; Ex. C, ¶24.)

There is no indication in the records that the lab results were communicated to the physician, nor is there any evidence that Leon's care was adjusted in response to the lab results.

(Plaintiffs' COE, Ex. B, ¶32; Ex. C, ¶25; Ex. E; Ex. J.) Plaintiffs' experts opine that the failure to communicate the results to a physician or adjust Leon's care was below the applicable standard of care and qualifies as neglect. (Plaintiffs' COE, Ex. B, ¶32; Ex. C, ¶25.)

Plaintiffs also submit evidence that Leon developed pressure wounds before December 13, 2020. Plaintiffs rely on the admission records for Leon completed at 1700 on December 12, 2020, which indicate multiple pressure wounds.

(Plaintiffs' COE, Ex. P, P000241-P000245.) Plaintiffs' expert opines to a reasonable degree of medical probability that Leon did not develop the pressure wounds developed less than 8 hours after being admitted to the Good Samaritan Hospital. (Plaintiffs' COE, Ex. C, ¶¶49-50.) There is therefore a triable issue of material fact as to whether the pressure wounds developed while Leon was in Defendants' care and custody.

Plaintiffs also establish that there was no care plan established for Leon to prevent pressure wounds despite being an identified concern for Leon. Defendants do not claim that such a care plan was ever created or implemented.

Plaintiffs submit evidence that Leon was deemed to be at risk for pressure injuries, but there was no plan for a "turning/repositioning program" created or implemented for Leon. (Plaintiffs' COE, Ex. B, ¶8(m); Ex. C, ¶10(m); Ex. L.)

"Neglect" includes, but is not limited to, all of the following: (1) Failure to assist in personal hygiene, or in the provision of food, clothing, or shelter. (2)

Failure to provide medical care for physical and mental health needs...(3)

Failure to protect from health and safety hazards. (4) Failure to prevent malnutrition or dehydration...." (Welf.

& Inst. Code, §15610.57, subd. (b)(1)-(4).)

Plaintiffs' evidence establishes Defendants' neglect based on their failure to attend to Leon's basic needs, including feeding and hydrating her in accordance with the physician order issued on December 3, 2020, preventing dehydration and preventing the development of pressure wounds due to lack of movement. Defendants argue on reply that these failures are failures to provide adequate medical care, not the failure to provide for Leon's basic needs. The court disagrees. Reasonable minds could differ on whether such failures qualify as "the failure of those responsible for attending to the basic needs and comforts of elderly or dependent adults, regardless of their professional standing, to carry out their custodial obligations." (Covenant Care, Inc., supra, 32 Cal.4th at 783.)

The court finds that Defendants have not met their burden of showing that the first cause of action for elder abuse has no merit, because Defendant has not shown that the element of neglect cannot be established. The court therefore denies Defendants' (1) motion for summary judgment and (2) motion for summary adjudication of the first cause of action for elder abuse.

b. Ratification

"[T]he standards set forth in subdivision (b) of Section 3294 of the Civil Code regarding the imposition of punitive damages on an employer based upon the acts of an employee shall be satisfied before any damages or attorney's fees permitted under this section may be imposed against an employer." (Welf. & Inst. Code, § 15657 (c).) As Defendants acknowledge on reply, ratification is therefore a necessary element to obtain punitive damages under Welfare and Institutions Code section 15657 subdivision (c). Ratification is therefore not an essential element of Plaintiffs' elder abuse claim and not grounds for summary adjudication of the elder abuse claim.

Moreover, Defendants fail to satisfy its burden as moving party even if ratification were an element of elder abuse. A party moving for summary judgment can establish that a cause of action has no merit if "one or more of the elements

of the cause of action cannot be separately established.” (Code of Civ. Proc., §437c, subd. (o)(1).) A defendant may satisfy their burden on

summary adjudication pursuant to Code of Civil Procedure section 437c

subdivision (o)(1) by showing that the claim “cannot be established” because of

the lack of evidence on some essential element of the claim. (Union Bank v. Superior Court (1995)

31 Cal.App.4th 574, 590.) A defendant

does not meet its burden under Code of Civil Procedure section 437c subdivision

(o)(1) by merely “pointing out” or “arguing” that the plaintiff does not

possess or cannot reasonably obtain necessary evidence.

Defendants failed to submit any evidence regarding ratification. Defendants cannot satisfy their burden by

simply stating that “Plaintiffs cannot prove that any officer, director or

managing agent of Defendant ratified its agents or employees’ alleged

intentional misconduct.” (Motion, 19:17,

20:1-2.)

The court finds that Defendants have not met their burden of

showing that the first cause of action for elder abuse has no merit, because

Defendants have not shown ratification is an element of elder abuse or that

Plaintiffs cannot establish ratification.

The court therefore also denies Defendants’ motion for summary

adjudication of the first cause of action for elder abuse on this ground.

2. Fifth

Cause of Action for Violations of Residents Rights (pursuant to Health & Safety Code Section 1430(b))

Health and Safety Code section 1430(b) expressly authorizes a

current or former resident or patient of a skilled nursing facility to bring a

civil action against the licensee of a facility who violates any rights of the

resident or patient as set forth in the Patient’s Bill of Rights in 22 California

Code of Regulations section 72527, or any other right provided for by federal

or state law or regulation.

Plaintiffs allege Leon’s patient rights under Title 22 of

California Code of Regulations section 72527, Health & Safety Code §1599.1,

Title 42 of the Code of Federal Regulations sections 483.10, 483.13, 483.15,

483.20 and other state and federal laws and regulations. (Complaint, ¶¶118-119.) These rights include the right to be given

care to prevent pressure sores (¶118(b)(iii), (v)), the right to be informed

and to have her physician informed of any need to alter treatment (¶118(c)(ix)),

the right to be cared for in a manner and in an environment that promotes maintenance or enhancement of Leon's life (§118(e)(i)), the right to receive necessary care and services to attain or maintain the highest practicable physical, mental, and psychosocial well-being, in accordance with comprehensive assessment and plan of care (§118(g)), the right to receive sufficient fluid intake to maintain proper hydration and health (§118(g)(viii)), and the right to a patient care plan and implementation of that plan (§118(j)).

Defendants discuss the causes of action for elder abuse cause and violation of residents rights per Health & Safety Code Section 1430(b) together. Defendants rely on the exact same arguments and evidence in moving for summary adjudication of both causes of action. For the same reasons discussed above in connection with the first cause of action for elder abuse, the court finds that Defendants have not met their burden of showing that the fifth cause of action for violation of residents rights pursuant to Health & Safety Code Section 1430(b) has no merit, because Defendants have not shown that the element of a violation of Leon's rights cannot be established.

3. Second

Cause of action for Negligence

Defendants' Notice of Motion does not include the second cause of action for negligence among the issues to be adjudicated. Defendants' Separate Statement lists several issues for adjudication that were not listed in the Notice of Motion: "Issue 2: Defendants did not neglect Perla Susana Leon," "Issue 3: Defendants' Care and Treatment of Perla Susana Leon at all times, complied with the standard of care," and "Issue No. 4: Defendants did not cause or substantially contribute to Perla Susana Leon's Alleged Injuries or Death." "If summary adjudication is sought, whether separately or as an alternative to the motion for summary judgment, the specific cause of action, affirmative defense, claims for damages, or issues of duty must be stated specifically in the notice of motion and be repeated, verbatim, in the separate statement of undisputed material facts." (Cal. Rules of Court Rule 3.1350, subd. (b).) By failing to include these issues in the Notice of Motion, Plaintiffs were not given notice that they were to be adjudicated and addressing them on their merits would be a violation of due process.

Defendants did not move for adjudication of the second cause of action for negligence. The second cause of action for negligence is therefore outside the scope of the motion and not subject to adjudication.

4. Fourth

Cause of Action for Wrongful Death

The elements of a wrongful death action are a tort (negligence or other wrongful act), resulting death (causation), and loss (damages) suffered by the heirs. When the tort is negligence, plaintiff must, of course, prove defendant's duty and breach thereof. (*Lattimore v. Dickey* (2015) 239 Cal.App.4th 959, 968.)

The elements of a medical malpractice cause of action are (1) the duty of the professional to use such skill, prudence, and diligence as other members of his profession commonly possess and exercise; (2) a breach of that duty; (3) a proximate causal connection between the negligent conduct and the resulting injury; and (4) actual loss or damage resulting from the professional's negligence. (*Avivi v. Centro Medico Urgente Medical Center* (2008) 159 Cal.App.4th 463, 468 fn 2.)

The standard of skill, knowledge and care prevailing in a medical community is ordinarily a matter within the knowledge of experts. (*Jambazian v. Borden* (1994) 25 Cal.App.4th 836, 844.)

"The standard of care for physicians is the reasonable degree of skill, knowledge and care ordinarily possessed and exercised by members of the medical profession under similar circumstances.

The test for determining familiarity with the standard of care is knowledge of similar conditions.

Geographical location may be a factor considered in making that determination, but, by itself, does not provide a practical basis for measuring similar circumstances." (*Avivi v. Centro Medico Urgente Medical Center*, *supra*, at 159 Cal.App.4th at p. 470.)

Whether the standard of care in the community has been breached presents a basic issue of fact in a malpractice action which can only be proved by expert opinion testimony unless the medical question is within the common knowledge of laypersons. (*Id.* at p. 470, fn. 1; see also 1 Witkin, *Cal. Evid.* (6th ed. 2021), Opinion, §52.) "[A]n opinion unsupported by reasons or explanations does not establish the absence of a material fact issue for trial, as required for summary judgment." (*Golden Eagle Refinery Co., Inc. v. Associated Intern. Ins. Co.* (2001) 85

Cal.App.4th 1300, 1315 (citing Kelley v. Trunk (1998) 66 Cal.App.4th 519, 524). Accordingly, where a plaintiff provides proper opposing expert testimony by a declaration, a triable issue of material fact is raised which precludes summary judgment. (Jambazian, supra, 25 Cal.App.4th at p. 844.) The absence of opinion evidence on this issue is fatal to the plaintiff's cause of action. (Willard v. Hagemeister (1981) 121 Cal.App.3d 406, 412.) An expert declaration is admissible as long as Defendants' expert states in detail their qualifications and the factual basis for their opinion. (Kelley, supra, 66 Cal.App.4th at p. 524.)

Expert testimony is also required to establish the element of causation in a medical malpractice action. "The law is well settled that in a personal injury action causation must be proven within a reasonable medical probability based on competent expert testimony. Mere possibility alone is insufficient to establish a prima facie case. That there is a distinction between a reasonable medical probability and a medical possibility needs little discussion. There can be many possible causes, indeed, an infinite number of circumstances that can produce an injury or disease. A possible cause only becomes probable when, in the absence of other reasonable causal explanations, it becomes more likely than not that the injury was a result of its action. This is the outer limit of inference upon which an issue may be submitted to the jury." (Jennings v. Palomar Pomerado Health Systems, Inc. (2003) 114 Cal.App.4th 1108, 1118 (trial court properly struck medical expert testimony on issue of causation where expert's opinion failed to provide a reasoned explanation regarding the etiology connecting plaintiff's infected subcutaneous tissue to the nidus inside peritoneal cavity where retractor was retained)).

Defendants' Notice of Motion does include the wrongful death cause of action among the issues to be adjudicated. The separate statement, however, does not specifically state the fifth cause of action wrongful death and the material facts that are relevant to the wrongful death claim are not identified. Defendants' motion for summary adjudication of the fifth cause of action for wrongful death may be denied on procedural grounds at the court's discretion based on Defendants' failure to comply with rule 3.1350. (Truong v. Glasser (2009) 181 Cal.App.4th 102, 118.)

The motion for summary adjudication must also be denied as to the fourth cause of action for wrongful death on

substantive grounds. As discussed in connection with the first cause of action for elder abuse and the fifth cause of action for violation of patient rights under Health and Safety Code section 1430, subd. (b), a triable issue of material fact exists as to whether Leon was neglected. Both parties submit competing expert declarations regarding Defendants' alleged negligence and whether Defendants' treatment and care of Leon was within the applicable standard of care. (Defendants' Evidence, Hibbs Dec., ¶¶48, 52, 53, 60; Rosen Dec., ¶¶48, 53, 61; Plaintiffs' COE, Ex. B, ¶¶10-50; Ex. C, ¶¶19, 23-26, 39-40, 44-50.) The existence of dueling expert opinions on the essential elements of breach and causation precludes summary adjudication of the wrongful death cause of action, which incorporates the elements of Plaintiffs' negligence claim.

The court finds that Defendants have not met their burden of showing that the fourth cause of action for wrongful death has no merit because Defendants have not shown that negligence cannot be established. Even if Defendants had met their burden of showing that the fourth cause of action for wrongful death has no merit because Defendants had shown that negligence cannot be established with expert evidence, the court finds that Plaintiffs have met their burden to show that a triable issue of material fact exists as to the element of Defendants' negligence. (Defendants' Evidence, Hibbs Dec., ¶¶48, 52, 53, 60; Rosen Dec., ¶¶48, 53, 61; Plaintiffs' COE, Ex. B, ¶¶10-50; Ex. C, ¶¶19, 23-26, 39-40, 44-50.)

The court therefore denies Defendants' motion for summary adjudication as to the fourth cause of action for wrongful death.

5. Third

Cause of Action for Negligent Hiring, Supervision and Retention (CACI 426)

The elements of a claim for negligent hiring, supervision or retention are set forth in CACI 426. The elements are (1) employer defendant hired/supervised/retained an employee; (2) the employee was or became unfit or incompetent to perform work for which the employee was hired; (3) employer defendant knew or should have known that employee was or became unfit or incompetent and that this unfitness or incompetence created a particular risk to other; (4) that the employee's unfitness or incompetence harmed plaintiff; and (5) employer defendant's negligence in hiring, supervising or retaining the employee was a substantial

factor in causing plaintiff's harm. (CACI 426.)

"California case law recognizes the theory that an employer can be liable to a third person for negligently hiring, supervising, or retaining an unfit employee." (Doe v. Capital Cities (1996) 50 Cal.App.4th 1038, 1054.)

Defendants rely on the exact same material facts submitted in support of the motion for summary adjudication of the first cause of action for elder abuse to satisfy their burden on summary adjudication of the negligent hiring, retention and supervision cause of action. (Defendant's Separate Statement, Issue 1, UMF Nos. 1-60; Issue 5, UMF Nos. 1-60.)

Defendants argue they are entitled to summary adjudication of the third cause of action for negligent hiring, retention and supervision because their experts opine that the care provided to Leon was within the applicable standard of care and, without evidence of incompetent care by the nurses, there is no basis to find negligent hiring, supervision or retention.

For the reasons discussed above in connection with the first cause of action for elder abuse, the court finds Defendants have not met their burden of showing that the third cause of action for negligent hiring, supervision and retention has no merit because Defendants have not shown that the element of negligence cannot be established. Even if Defendants had met their burden of showing that the third cause of action has no merit because the element of negligence cannot be established, the court finds that Plaintiffs have met their burden to show that a triable issue of material fact exists as to the element of Defendant's negligence for the reasons set forth above.

6. Punitive Damages

"Under the clear and convincing standard, the evidence must be so clear as to leave no substantial doubt and sufficiently strong to command the unhesitating assent of every reasonable mind. Although the clear and convincing evidentiary standard is a stringent one, it does not impose on a plaintiff the obligation to 'prove' a case for punitive damages at summary judgment or summary adjudication. Even

so, where the plaintiff's ultimate burden of proof will be by clear and convincing evidence, the higher standard of proof must be taken into account in ruling on a motion for summary judgment or summary adjudication, since if a plaintiff is to prevail on a claim for punitive damages, it will be necessary that the evidence presented meet the higher evidentiary standard. Likewise, in reviewing the trial court's denial of PG&E's motion for summary adjudication, we view the evidence with the higher burden of proof in mind.

Summary judgment or summary adjudication on the issue of punitive damages is proper only when no reasonable jury could find the plaintiff's evidence to be clear and convincing proof of malice, fraud or oppression." (Pacific Gas and Electric Company v. Superior Court (2018) 24 Cal.App.5th 1150, 1158-1159.)

Malice is conduct that is intended by the defendant to cause the plaintiff harm or despicable conduct carried out with a willful and conscious disregard of the rights or safety of that person. (Civ. Code, §3294, subd. (c).) "'Recklessness' refers to a subjective state of culpability greater than simple negligence, which has been described as a 'deliberate disregard' of the 'high degree of probability' that an injury will occur. Recklessness, unlike negligence, involves more than 'inadvertence, incompetence, unskillfulness, or a failure to take precautions' but rather rises to the level of a 'conscious choice of a course of action ... with knowledge of the serious danger to others involved in it.'" (Delaney, supra, 20 Cal.4th at pp. 31-32.)

Defendant's motion for summary adjudication of the punitive damages claim is based on the exact same 60 material facts submitted in support of the first cause of action for elder abuse.

For the same reasons stated in connection with the elder abuse cause of action, Defendant fails to negate Plaintiff's allegation that Defendants knowingly neglected Leon and engaged in despicable conduct carried out with willful and conscious disregard of Leon's safety and well-being.

(Complaint, ¶¶75, 76, 79, 102.)

Defendants failed to establish as an issue of law that they provided for Leon's basic needs, including hydration and prevention of bed sores, and any failure to do so was not the result of despicable conduct carried out with willful and conscious disregard of Leon's safety and well-being.

The court finds that

Defendants have not met their burden of showing that Plaintiffs' punitive damages claim has no merit because Defendants have not shown that the element of malice cannot be established.

The court therefore denies Defendants' motion for summary adjudication as to the punitive damages claim.

ORDER

The court denies Defendant's motion for summary judgment or, in the alternative, summary adjudication.

The court orders Plaintiffs to give notice of this ruling.

IT IS SO ORDERED.

DATED: July 21, 2026

Robert
B. Broadbelt III

Judge
of the Superior Court